

Q4 2025 Earnings Call

Company Participants

- Elizabeth Shea, Senior Vice President, Investor Relations
- Jeff Stewart, Executive Vice President, Chief Commercial Officer
- Robert A. Michael, Chairman of the Board and Chief Executive Officer
- Roopal Thakkar, Executive Vice President, Research & Development, Chief Scientific Officer
- Scott T. Reents, Executive Vice President, Chief Financial Officer

Other Participants

- Asad Haider, Goldman Sachs
- Chris Schott, J.P. Morgan
- David Amsellem, Piper Sandler
- David Risinger, Leerink Partners
- Luisa Hector, Berenberg
- Malcolm Hoffman, BMO Capital Markets
- Michael Yee, UBS
- Mohit Bansal, Wells Fargo
- Steve Scala, TD Cowen
- Terence Flynn, Morgan Stanley
- Vamil Divan, Guggenheim Securities

Presentation

Operator

Good morning, and thank you for standing by. Welcome to the AbbVie Fourth Quarter 2025 Earnings Conference Call. All participants will be able to listen-only until the question-and-answer portion of this call. (Operator Instructions) Today's call is also being recorded. If you have any objections, you may disconnect at this time.

I would now like to introduce Ms.Liz Shea, Senior Vice President, Investor Relations.

Elizabeth Shea {[BIO 18811464 <GO>](#)}

Good morning, and thanks for joining us. Also, on the call with me today are Rob Michael, Chairman and Chief Executive Officer; Jeff Stewart, Executive Vice President, Chief Commercial Officer; Roopal Thakkar, Executive Vice President, Research and Development, Chief Scientific Officer; and Scott Reents, Executive Vice President, Chief Financial Officer.

Before we get started, I'll note that some statements we make today may be considered forward-looking statements based on our current expectations. AbbVie cautions that these forward-looking statements are subject to risks and uncertainties that may cause actual results to differ materially from those indicated in our forward-looking statements. Additional information about these risks and uncertainties is included in our SEC filings. AbbVie undertakes no obligation to update these forward-looking statements except as required by law.

On today's conference call, non-GAAP financial measures will be used to help investors understand AbbVie's business performance. These non-GAAP financial measures are reconciled with comparable GAAP financial measures in our earnings release and regulatory filings from

today, which can be found on our website. Following our prepared remarks, we will take your questions.

So with that, I'll turn the call over to Rob.

Robert A. Michael {BIO 19541407 <GO>}

Thank you, Liz. Good morning, everyone, and thank you for joining us. Our fourth quarter performance closes out another excellent year for AbbVie, and I am very pleased with the significant progress we made in 2025.

We delivered record net sales and exceeded our financial commitments, advanced our pipeline across all stages of development, and acquired new sources of growth through strategic transactions. We are entering 2026 with substantial momentum and remain well-positioned to deliver another year of strong growth.

Turning to our results. We delivered full-year adjusted earnings per share of \$10, which is \$0.54 above our initial guidance midpoint, excluding the impact of IPR&D expense. Total net revenues were \$61.2 billion, beating our initial guidance by more than \$2 billion.

Our sales growth of 8.6% led to a new all-time high for AbbVie, exceeding our previous peak revenue by more than \$3 billion, despite nearly \$16 billion of U.S. Humira erosion since the LOE. Our excellent performance puts us in a strong position to advance our innovative medicines.

In 2025, we increased adjusted R&D expense by nearly \$1 billion, fully funding the 90 clinical programs currently in development. We also obtained several new product and indication approvals, including Rinvoq for GCA, EMRELIS for non-squamous, non-small cell lung cancer, and Epkinly for second-line follicular lymphoma. And we bolstered our pipeline by investing more than \$5 billion in new business development, including several promising mechanisms and technologies.

These include an in vivo CAR-T platform in immunology from Capstan Therapeutics; Bretisilocin, a next-generation psychedelic with promising data in depression; ISB 2001, a novel trispecific antibody for multiple myeloma; 295, a long-acting amylin analog for obesity; and a next-generation siRNA platform from ADARx that has applicability in immunology, neuroscience, and oncology.

We also recently announced a transaction with RemeGen, adding a novel PD-1/VEGF bispecific antibody to combine with our ADCs across multiple solid tumors, further strengthening AbbVie's oncology portfolio.

Turning to 2026. We expect AbbVie to once again deliver top-tier performance. We anticipate total sales growth of 9.5%, reflecting another year of robust sales results despite headwinds from continued Humira erosion and Imbruvica IRA pricing. The main drivers of this growth include Skyrizi and Rinvoq with combined sales of more than \$31 billion, already surpassing our 2027 long-term guidance by \$0.5 billion.

We are also forecasting a substantial sales ramp for Vyalev, achieving blockbuster status this year as a transformational treatment for Parkinson's, and we expect continued double-digit revenue growth from our leading migraine products, which are also trending significantly above our long-term expectations. Given the strong outlook of our diverse portfolio, we are well-positioned to deliver high single-digit revenue growth through 2029.

Turning now to R&D. We anticipate a number of key catalysts across our core therapeutic areas over the next 24 months. In immunology, this includes pivotal data for three additional Rinvoq indications, as well as initial data for our Crohn's combination platform with Skyrizi. In neuroscience, we anticipate key readouts for next-generation assets 932, bretsilocin, and emraclidine.

And in oncology, we expect registrational data for etentamig, as well as mid to late-stage readouts for Temab-A in several solid tumors. These are all very exciting opportunities that have the potential to drive sustained long-term growth.

Lastly, we recently announced a voluntary agreement with the U.S. government that reinforces our commitment to patient access and affordability, while also protecting our ability to invest in innovation. Key elements of this three-year agreement include offering low prices in Medicaid, expanding direct-to-patient cash pay options for select products, and committing \$100 billion in U.S. R&D and capital investments over the next decade. In summary, we are delivering outstanding execution, and the outlook of our business remains very strong.

With that, I'll turn the call over to Jeff for additional comments on our commercial highlights. Jeff?

Jeff Stewart {[BIO 19485431](#) <GO>}

Thank you, Rob. I'll start with the quarterly results for immunology, which delivered total revenues of approximately \$8.6 billion. Skyrizi total sales were \$5 billion, reflecting operational growth of 31.9%. Rinvoq total sales were nearly \$2.4 billion, reflecting operational growth of 28.6%.

On a full-year basis, Skyrizi and Rinvoq delivered approximately \$25.9 billion in total combined revenue, an impressive increase of more than \$8 billion year-over-year, well exceeding our initial guidance expectations. These results reflect exceptional performance across their respective indications, and I'll highlight a few examples.

Starting with psoriatic disease, a market that is growing high single digits with modest biologic penetration where our portfolio has clear leadership. Skyrizi total prescription share in the U.S. biologic psoriasis market is now more than 45% and accelerating in the fourth quarter. Our in-play capture rates of new and switching patients has exceeded 55% across all lines of therapy, 4x higher than the next closest competitor.

And when you consider Skyrizi's very high and durable skin clearance, including new and statistically significant data across the board for hard to treat areas like scalp, genitals, palmar, plantar, also widely demonstrated superior efficacy to both biologic and to oral agents, along with that simple quarterly dosing, we do not expect a material impact to our robust outlook in psoriasis from any existing or new therapies this year and beyond.

Moving to PsA Derm, where nearly 30% of patients visiting dermatologists have both skin and joint involvement. Skyrizi is now capturing roughly one out of every four in-play patients on biologics across all lines of therapy in the U.S., further supporting Skyrizi's strong momentum.

The PsA indication represents a critical differentiator in the psoriasis market, especially relative to emerging options where PsA efficacy is still unproven. Importantly, Skyrizi has achieved PsA frontline in-play patient shared leadership for biologics in both the Derm and the Rum segments.

Turning more broadly to RUM. Rinvoq continues to achieve a leading mid-teen in-play patient share for RA across all lines of therapy in the U.S. This is roughly double our total TRx share, highlighting a nice setup for incremental share capture in RA over the next several years, driven

by our active communication of the SELECT-SWITCH trial, with data demonstrating Rinvoq's doubling of remission rates versus treatment with a second TNF inhibitor.

And lastly, in IBD, where we continue to deliver strong performance and remain very well-positioned. The IBD market is very robust, with high single-digit growth, driven by increasing biologic penetration and rapidly expanding lines of therapy as patients cycle to newer, high-efficacy agents like Skyrizi and Rinvoq. We are very pleased with Skyrizi's strong growth in IBD.

Global sales for this indication were approximately \$6.4 billion in 2025, more than double our prior year performance in IBD. We remain very we remain very confident in Skyrizi's profile in IBD, including its demonstrated strong impact on clinical remission as well as extremely strong endoscopic data with best-in-class placebo adjusted rates particularly in the bio-naive patients. And you can see this clinical profile playing out well when you consider our current market shares by line of therapy.

Importantly, and despite in-class competition for most of 2025, Skyrizi's capture rates remain exceptionally impressive, especially in that frontline treatment of IBD, which is the strongest signal of overall physician preference, and Skyrizi remains the clear leader.

Skyrizi retains a very high IL-23 category patient share with an in-play capture rate of approximately 75% in the frontline setting overall for IBD. This is driven by an even higher frontline capture rate for Skyrizi and Crohn's disease, which is roughly two-thirds of the total IBD market. And these capture rates have been strong and consistent even as the IL-23 category expands rapidly.

Skyrizi's dosing convenience is also favored with less frequent maintenance treatment to the most effective dose for other IL-23s, which is very important as patients tend to be on therapy for many years. So, Skyrizi continues to perform very well and will continue to do so in 2026 and beyond.

Equally importantly, and unlike any other competitor, we have a second compelling treatment in IBD, Rinvoq, which is also capturing robust mid-teens in-play share across all lines of therapy in Crohn's disease and ulcerative colitis.

Rinvoq has demonstrated some of the strongest response rates to-date in IBD, including rapidity of action, which is important for patients who need rapid control and durable remission. And with Rinvoq's recently expanded label in IBD, patients will now have access to Rinvoq earlier in the treatment paradigm when anti-TNF treatment is clinically inadvisable.

Skyrizi and Rinvoq are a great pair in IBD. Skyrizi is well-positioned in frontline, and we see more opportunity than ever before for Rinvoq in the second-line plus setting. So together, our two brands have already exceeded peak Humira sales by more than \$4.5 billion and are on pace to deliver more than 20% growth in 2026, remarkable considering this year will be their eighth year on the market.

Turning now to Humira, which delivered global sales of more than \$1.2 billion, down 26.1% on an operational basis, primarily due to biosimilar competition, and in line with our expectations. We anticipate Humira access will decrease further throughout 2026 as more plans move to exclusive biosimilar contracts.

Moving to neuroscience, where full-year revenues were more than \$10.7 billion, reflecting impressive absolute sales growth of nearly \$1.8 billion. In the quarter, total revenues were more than \$2.9 billion, up 17.3% on an operational basis.

This robust performance is driven by continued double-digit growth of Vraylar, with global sales of \$1 billion; Botox Therapeutic, with global revenues of \$990 million; Ubrelevy, with global sales of \$339 million; and Qulipta, with global revenues of \$288 million. Beyond these leading therapies for psychiatry and migraine, we are very excited for our emerging portfolio in Parkinson's disease, which we believe remains underappreciated.

Vyalev's launch continues to be outstanding. Total sales were \$183 million in the quarter, up approximately 33% on a sequential basis. The uptake is exceptionally strong across international markets, and we expect sales to ramp in the U.S., where Vyalev recently received full coverage. Feedback from prescribers and patients communities remain very encouraging, highlighting meaningful improvements in on-time and off-time as a result of Vyalev's continuous 24-hour delivery and the control of symptoms morning, day, and night.

Given these insights and the robust early launch trends globally, we now expect Vyalev to achieve blockbuster revenue in 2026. And when you add Tavapadon for potential use as a monotherapy for early Parkinson's disease, as well as an adjunct to optimize oral therapy for more advanced patients, we believe we have the potential multi-billion-dollar emerging PD franchise over the long-term. And we remain on track for commercial approval of Tavapadon in the U.S. later this year.

Moving now to oncology, where total revenues were nearly \$1.7 billion in the quarter, down 2.5% on an operational basis. Venclexta global sales were \$710 million, up 6.4% on an operational basis, reflecting continued strong demand in CLL, with combination use of Venclexta plus BTK inhibitors emerging as a preferred all-oral fixed-duration treatment.

In 2026, we anticipate another major commercial catalyst, with the global approvals of Venclexta plus Calquence in combination, two leading brands in CLL, offering patients the potential for time-off treatment, addressing an important need.

Double-digit sales growth from Elahere, Epkinly, and EMRELIS also helped to partially offset the expected sales decline for Imbruvica, which was down 20.8%, primarily due to continued competitive dynamics. And we do anticipate Imbruvica IRA pricing will unfavorably impact our oncology portfolio growth in 2026.

Turning now to aesthetics, which delivered global sales of nearly \$1.3 billion in the quarter, down 1.2% on an operational basis. Botox Cosmetic global revenues were \$717 million, up 3.8% on an operational basis. Juvederm global sales were \$249 million, down 10.8% on an operational basis.

As we've seen over the last several quarters, economic headwinds have continued to impact market conditions globally and we anticipate category growth will remain challenged in 2026. With our leading market shares relatively stable for both toxins and fillers we are focused on investing to stimulate the market, which remains highly underpenetrated.

We expect a further catalyzed growth with new promotional programs for botox, including the Only You campaign that was launched over the last several months with encouraging early results, and an unbranded program to educate practices and consumers about the benefit of HA fillers with a focus on driving natural outcomes. As well as additional injector training, which will be supported by our three new AMI training centers in the U.S. as well as training programs in key international geographies.

Bringing innovation to the aesthetics market with our pipeline is also a clear priority. We look forward to commercializing TrenibotE, a fast-acting short-duration toxin, which is expected to be approved in the U.S. later this year. So overall, I'm extremely pleased with the execution across

our commercial portfolio, which is demonstrating very, very strong momentum as we head into 2026.

And with that, I will turn the call over to Roopal for comments on our R&D highlights. Roopal?

Roopal Thakkar {[BIO 23690312 <GO>](#)}

Thank you, Jeff. I'll start with immunology, where we are on track for numerous important data readouts across all stages of our pipeline this year, as well as several regulatory submissions and approvals.

Regulatory applications for Rinvoq in non-segmental vitiligo were recently submitted with an approval decision in the U.S. anticipated in the fourth quarter. Regulatory applications for Rinvoq in alopecia areata are under review in Europe and Japan, with approval decisions expected later this year. Our U.S. submission is planned for the second quarter.

Over the course of 2026, results from several late-stage programs are anticipated, including Phase 3 data from studies for both Rinvoq and Lutikizumab in hidradenitis suppurativa and a study evaluating subcutaneous induction with Skyrizi in Crohn's disease. We continue to make substantial progress with our early and mid-stage programs as well, where we have an exciting pipeline of next-generation therapies that have the potential to drive higher efficacy relative to standard of care.

This year, we'll see data from our Crohn's disease platform study, evaluating Skyrizi in combination with our novel anti-alpha-4/beta-7 antibody, ABBV-382, and our anti-IL-1-alpha-beta bispecific lutikizumab. Our extended half-life TL1A antibody will also be evaluated in combination with Skyrizi, with a Phase 2b dose-ranging study in Crohn's and ulcerative colitis beginning later this year.

Separately, a Phase 2 study is underway to evaluate our TREM1 antibody as a monotherapy in Crohn's disease. Data from this study will be available later this year and will help inform our combination strategy for this molecule. The Phase 1b trial for ABBV-319, our anti-CD19 ADC with a steroid payload, is now underway.

And we will soon begin dosing patients with our in vivo CD19 CAR-T, ABBV-619. These B-cell depleting approaches have the potential to become transformative modalities to reset the immune system and provide deep, durable, drug-free remission for patients with autoimmune diseases.

Individual patient data from dose-escalation studies will be available on a rolling basis, and we anticipate seeing efficacy results later this year. Several additional immunology assets will also be entering the clinic this year, including an extended half-life anti-IL-23 antibody and an oral peptide IL-23 inhibitor.

Moving to oncology. Progress continues with our next-generation c-Met ADC Temab-A. Strong data have been observed in late-line colorectal cancer as both in monotherapy and in combination with bevacizumab. Therefore, we will be initiating a Phase 3 study this year in an all-comers population in combination with bev.

Dose optimization continues for Temab-A in non-small cell lung cancer, where both EGFR wild-type and EGFR-mutant populations are being evaluated. Data from these studies will be available next year, informing our Phase 3 path. In pancreatic cancer, Temab-A will be studied in combination with different regimens of chemotherapy. Later this year, data is anticipated in head and neck and ovarian cancer.

We're making very good progress across several tumor types with Temab-A. We recently announced a deal with RemeGen for a PD-1/VEGF antibody. This molecule will be a nice complement to our ADC portfolio and has demonstrated competitive monotherapy efficacy, as well as encouraging early data in ADC and chemo combinations.

Our strategy is to initially combine this PD-1/VEGF with Temab-A in lung and colorectal, with other tumor types also under consideration. These novel combinations have the potential to drive faster disease control, longer duration, and ultimately longer survival.

In small cell lung cancer, a Phase 2 trial for ABBV-706 in combination with atezolizumab recently began in treatment-naïve patients. A Phase 3 study in second-line plus patients is also planned to initiate this year. In hematologic malignancies, the regulatory application for PVEK in blastic plasmacytoid dendritic cell neoplasm is under review with the FDA, and approval decision is anticipated later this year.

Enrollment is projected to complete this quarter in our Phase 3 trial, evaluating monotherapy etentamig in third-line plus multiple myeloma with an objective response rate readout anticipated in the second half of this year. A Phase 2 study of etentamig plus daratumumab in front-line transplant in eligible patients was recently initiated. Additionally, a Phase 3 study in second-line evaluating etentamig with pomalidomide is expected to begin by early 2027.

Turning to neuroscience. In our movement disorder programs, an approval decision is anticipated in the third quarter for Tavapadon in Parkinson's disease. As a highly effective treatment for motor symptoms with low rates of dyskinesia, edema, sedation, and impulse control disorder, Tavapadon has the potential to be an important new treatment option, both as a monotherapy and as an adjunct to levo/carbidopa in patients still experiencing motor fluctuations.

This year, a Phase 2 study assessing Gemibot A in essential tremor will begin. Essential tremor is the most common movement disorder and an area with considerable need for effective and tolerable therapies. Unlike most patients with spasticity, patients with essential tremor typically have normal strength.

Therefore, it is important that neighboring muscles are not inadvertently affected, and this is by toxin diffusion. Our new toxin is highly potent and has the potential for less spread to neighboring muscles, making it well-suited for treating essential tremor.

In migraine, the Phase 3 ECLIPSE trial evaluating Qulipta for acute treatment of migraine met its primary and key secondary endpoints, with Qulipta demonstrating superiority over placebo. Approval decisions in Europe and Japan are expected later this year. Our Phase 3 studies evaluating Qulipta and Ubrelvy for menstrual migraine prevention are progressing well, with data from both trials expected in the second half of this year.

Regulatory submission for Qulipta in Europe is anticipated later this year and for Ubrelvy in the U.S. in 2027. Menstrual migraine is a distinct subtype affecting nearly 15% of women with migraine. Attacks are considered more difficult to treat, more disabling, last longer, and have a higher tendency to recur. There is a clear need for more effective treatment for this form of migraine.

In the area of psychiatry, several readouts and study starts are planned this year across multiple programs. Our dose-escalation study continues for emraclidine in schizophrenia, with the 75 milligram dose cleared and 100 milligrams currently being assessed. Further dose-escalation is planned until a tolerability threshold is reached.

Based on the favorable profile with 75 milligrams and the potential to move the dose even higher, emracridine will be moving forward in development as both a monotherapy and adjunctive treatment for schizophrenia.

Dose ranging in elderly patients is also ongoing, which will support development plans in psychosis related to Alzheimer's, Parkinson's, and dementia with Lewy bodies. Phase 2 studies across all indications will begin after the completion of multiple ascending dose evaluation. The Phase 2 study for ABBV-932 in bipolar depression is nearing completion, and data is expected around the middle of this year.

Data from the Generalized Anxiety Disorder Phase 2 is anticipated in the early part of 2027. This year, data from two additional cohorts from a Phase 2 study evaluating bretisiloin in major depressive disorder will help inform our development strategy and Phase 3 plans. This short-acting psychedelic demonstrated very strong and durable efficacy in a preliminary Phase 2 study. Based on this emerging profile, bretisiloin has the potential to become a groundbreaking new treatment in depression.

Moving to other areas of our pipeline. In obesity, data will be available this year from two ongoing Phase 1 studies evaluating our long-acting amylin analog, ABBV-295 in overweight and obese patients. Results will guide our Phase 2 program, which is expected to begin near the end of this year. In aesthetics, the regulatory application for our rapid-onset short-acting toxin, TrenibotE, is under review, and an approval decision is expected this year.

To summarize, we continue to demonstrate significant progress across all stages of our pipeline and anticipate many important regulatory and clinical milestones in 2026.

With that, I'll turn the call over to Scott.

Scott T. Reents {[BIO 21044531 <GO>](#)}

Thank you, Roopal. Starting with our fourth quarter results, we reported adjusted earnings per share of \$2.71, which is \$0.08 above our guidance midpoint. These results include a \$0.71 unfavorable impact from acquired IPR&D expense.

Total net revenues were \$16.6 billion, reflecting robust growth of 10%, including a 0.5% favorable impact from foreign exchange. Our ex-Humira growth platform delivered reported growth of 14.5%, once again exceeding our expectations. Adjusted gross margin was 83.6% of sales, adjusted R&D expense was 15.4% of sales, and adjusted SG&A expense was 22.3% of sales.

The adjusted operating margin was 38.3% of sales, which included a 7.6% unfavorable impact from acquired IPR&D expense. Net interest expense was \$655 million. The adjusted tax rate was 18.3%, reflecting the lower deductibility of the acquired IPR&D expense this quarter.

Turning to our financial outlook for 2026. Our full-year adjusted earnings per share guidance is between \$14.37 and \$14.57. Please note that this guidance does not include an estimate for acquired IPR&D expense that may be incurred throughout the year. We expect total net revenues of approximately \$67 billion, reflecting growth of 9.5%. At current rates, we expect foreign exchange to have a roughly 0.8% favorable impact on full-year sales growth.

This revenue forecast contemplates the following approximate assumptions for select key products and therapeutic areas. We expect global immunology sales of \$34.5 billion, including Skyrizi revenue of \$21.5 billion, driven by market growth and share gains across the psoriatic and IBD indications; Rinvoq revenue of \$10.1 billion, with growth across rum, dermatology, and gastro indications; and Humira total revenue of \$2.9 billion, reflecting continued biosimilar impact.

For neuroscience, we expect global sales of \$12.5 billion, reflecting robust double-digit growth. This includes Vraylar revenue of \$4 billion, driven by continued strong prescription demand; Botox Therapeutic sales of \$4.1 billion, with strong performance across indications; total oral CGRP revenue of \$2.9 billion, supported by market growth and share gains; and Vyalev sales of \$1 billion, demonstrating the brand's impressive launch and multi-billion-dollar potential.

In oncology, we expect global sales of \$6.5 billion, including Imbruvica revenue of \$2.2 billion, reflecting competitive headwinds and the impact of lower IRA-related pricing; Venclexta sales of \$3 billion, which reflects continued strong demand; and Elahere revenue of \$850 million.

For aesthetics, we expect global sales of \$5 billion. This includes Botox Cosmetic revenue of \$2.7 billion, reflecting modest market growth and relatively stable market share, and Juvederm sales of \$950 million, reflecting continued headwinds in key dermal filler markets.

Moving to the P&L for 2026. We are forecasting full-year adjusted gross margin above 84% of sales, adjusted R&D expense of approximately \$9.7 billion, and adjusted SG&A expense of approximately \$14.2 billion. We also expect an adjusted operating margin ratio of approximately 48.5%, reflecting meaningful expansion compared to 2025. We expect adjusted net interest expense of approximately \$2.8 billion, which includes anticipated refinancing.

We forecast our non-GAAP tax rate to be approximately 14%, reflecting a modest underlying improvement compared to 2025 due to recent tax law changes. Keep in mind that last year's non-GAAP tax rate of 18% included a 3% impact from acquired IPR&D expense. Finally, we expect our share count to be roughly in line with 2025.

Turning to the first quarter. We anticipate net revenues of approximately \$14.7 billion. At current rates, we expect foreign exchange to have a roughly 2% favorable impact on sales growth. This revenue forecast considers the following approximate assumptions for key products in selected therapeutic areas.

We expect global immunology sales to approach \$7.1 billion, including Skyrizi sales of \$4.4 billion and Rinvoq revenue of \$2 billion, reflecting typical seasonality as well as an unfavorable price comparison for Rinvoq related to timing of prior year rebates in the first half.

We also anticipate neuroscience revenue of \$2.8 billion, oncology sales of \$1.6 billion, and aesthetics revenue of \$1.2 billion, reflecting growth of roughly 9%, including a favorable comparison as we lap one-time price headwinds associated with changes to the Aly [ph] program.

We are forecasting an adjusted operating margin ratio of roughly 46% and model a non-GAAP tax rate of approximately 13.7%. We expect adjusted earnings per share between \$2.97 and \$3.01. This guidance does not include acquired IPR&D expense that may be incurred in the quarter.

Finally, AbbVie's robust business performance continues to support our capital allocation priorities. Our cash balance at the end of December was approximately \$5.2 billion, and we expect to generate free cash flow of approximately \$18.5 billion in 2026, which includes roughly \$3.5 billion of Skyrizi royalty payments. This free cash flow will support a strong and growing quarterly dividend, which we have increased by more than 330% since inception, as well as investments in business development to further enhance our portfolio.

In closing, we are pleased with AbbVie's results in 2025, and our financial outlook remains very strong. We have significant momentum across our diverse portfolio and are well-positioned to deliver strong results in 2026 and beyond.

With that, I'll turn the call back over to Liz.

Elizabeth Shea {BIO 18811464 <GO>}

Thanks, Scott. We will now open the call for questions. In the interest of hearing from as many analysts as possible over the remainder of the call, we ask that you please limit your questions to one or two.

Operator, we'll take the first question, please.

Questions And Answers

Operator

(Question And Answer)

Thank you. (Operator Instructions) The first question in the queue is from Dave Risinger with Leerink Partners. Your line is open.

Q - David Risinger {BIO 1504228 <GO>}

Thanks very much, and congrats on the solid fourth quarter and the momentum of the company. I just have one question, which is you mentioned the psychedelic that's in development. Could you please provide some more details on that, including the opportunity to improve upon the profile of J&J's Bravado? Thanks very much.

A - Roopal Thakkar {BIO 23690312 <GO>}

Hi. It's Roopal. I'll take that one. We see this one as a potential breakthrough type of therapy in a couple elements. One is it works through 5-HT2A serotonergic pathway and has a short-acting, I would say, hallucination time period where patients experience the potential hallucinations upfront, and then it rapidly goes away.

And what we've observed is that you see immediate efficacy, and then you see that efficacy held on for quite some time. So that's a benefit. And these patients are then able to stay for a short time in the clinic and then leave. The other notable differences from any other medications in this space are the very high levels of efficacy that were observed. And that almost led to a remission-like state in the majority of patients.

Now, we have two more readouts before the Phase 3 begins. And in those readouts, what we're doing is reducing the dose of the control arm. And recall, the data that we've seen thus far were not against placebo. They were, in fact, against an active control of the drug itself, which that data looked pretty good. But when you looked at the main dose, it looked very impressive.

So we're going to look at a comparator with a slightly lower dose of self-control, and then that will read out, I would say, by Q3 or so, and then we'll have a Phase 3 plan going forward that we'll discuss with regulators. But this has a potential to be highly differentiated.

A - Elizabeth Shea {BIO 18811464 <GO>}

Thanks, Dave.

Q - David Risinger {BIO 1504228 <GO>}

Great. Thank you.

A - Elizabeth Shea {BIO 18811464 <GO>}

Operator, next question, please.

Operator

Yes. The next question is from Chris Schott with J.P. Morgan. Your line is open.

Q - Chris Schott {BIO 6299911 <GO>}

Hi. Great. Thanks so much. Just a couple of parter on the immunology franchise. I guess, just first on Skyrizi in IBD. It sounds like you haven't really seen any change in new start share here with the Tremfya entry is my question, is that the case in both Crohn's and UC, or is that just more a holistic kind of IBD comment?

And my bigger picture question is, I've been reading more investors feeling that the Street now better understands the potential for Skyrizi and Rinvoq over time. As a result, there's no longer as much potential upside to numbers. I guess, I'm just interested in your view of consensus growth expectations for these assets over time. Is the market now largely getting this right at this point, or do you see further potential for growth upside as you think about the breadth of indications, et cetera? Thank you.

A - Jeff Stewart {BIO 19485431 <GO>}

Yes. Hi, Chris. It's Jeff. So, we see some slight distinctions between the capture rate for Skyrizi in Crohn's versus UC, and that's really probably largely based on when the launches took place with the competitors.

But as I mentioned, and it may not be widely understood, we see definitely a bifurcation in where the capture rate is coming. It's quite clear that our new patient starts are very, very stable, and they're very high in both UC and Crohn's.

If you look at where the starts are coming from, and I mentioned in my prepared remarks, we have very, very high capture rates, 75% overall in IBD in frontline setting, which sort of gives you a sense of where the preference of the customers are. And it's 80% -- I didn't highlight that in my speech, it's 80% in Crohn's, so it's slightly lower in UC on the frontline setting.

So we see the dynamics, again, is very stable and high new patient starts, despite the competitive entrant, high capture rates, leading capture rates by far in frontline, which is really, really critical, which gives you a sense the competitor is coming in and capturing in the second-line. And then overall, we see a very substantial overall IL-23 category expansion.

So when we look at all of those dynamics, our compete level is extremely high, and we're very, very comfortable with the momentum that we're going to continue to see with Skyrizi, even before we get to Rinvoq coming in, in the later line. So that gives you some sense overall on IBD. I'll let maybe Rob address the second question.

A - Robert A. Michael {BIO 19541407 <GO>}

Yes. So Chris, this is Rob. Thanks for your question. Look, I see numerous sources of upside across the enterprise. I'll start with your question on Skyrizi and Rinvoq.

Obviously, the combined guidance for this year is already \$0.5 billion higher than our 2027 estimate. And we do expect both Skyrizi and Rinvoq to grow robustly into the 2030s. And clearly,

as we look at models, that longer-term growth is not reflected. And so we do think on a longer-term basis, there's clearly more upside to Skyrizi and Rinvoq.

And then when you consider things like the market growth projections will gain share, along with the next wave of Rinvoq indications, we have a lot of confidence in the long-term potential for those two assets. But beyond immunology, when you look at AbbVie, I think what's underappreciated is both, I'd say neuroscience and oncology in particular. I'd say neuroscience clearly overperforming. We delivered nearly 20% growth last year. We expect to deliver mid-teens growth this year. It'll put us in the number one position in the industry.

I mean, Vyalev has already -- will already achieve our peak sales guidance this year. We had given long-term guidance of greater than \$1 billion peak. We're guiding to \$1 billion. We think our Parkinson's franchise, including Vyalev, Duopa, and Tavapadon can achieve peak sales in excess of \$5 billion. That's not reflected in treatment models.

And given the ramp of our oral CGRPs, we believe our migraine franchise will also exceed \$5 billion in peak sales. Recall, we gave peak sales estimates of greater than \$3 billion based on this year's guidance. We're almost there. And so, clearly, the Street is not modeling that type of upside for our migraine business.

In psychiatry, we have several next-generation assets with 932, bretisilocin, as Roopal highlighted, and emraclidine, which we're clearly studying the dose. That can help replace the \$5 billion peak of Vraylar after its LOE in 2030. So, we're in a very strong position to lead in neuroscience for the long-term with three \$5 billion-plus franchises.

And I haven't even mentioned essential tremor or other therapeutic toxin indications or the opportunity in Alzheimer's. And so, we have, I'd say, profound opportunity to lead in neuroscience for the long-term, and that's clearly not reflected in Street models.

And then when I look at oncology, we have a very compelling pipeline that doesn't get enough attention. For example, our ADC, Temab-A has shown promising data in CRC, lung, gastroesophageal, and pancreatic cancers. In CRC alone, Temab-A has the potential to be a multi-billion-dollar opportunity, not to mention the other solid tumors that it could potentially treat.

And we also have exciting opportunities in oncology, such as etentamig and the follow-on trispecifics and multiple myeloma, 706 in small cell lung cancer, and 969 in prostate cancer. So we have many opportunities to deliver upside to Street long-term estimates. And with no significant LOEs until the next decade, we have plenty of capacity to invest in both internal and external innovation to drive very strong long-term growth.

Q - Chris Schott {BIO 6299911 <GO>}

Thank you.

A - Elizabeth Shea {BIO 18811464 <GO>}

Thank you, Chris. Operator, next question, please.

Operator

Next question is from Terence Flynn with Morgan Stanley. Your line is open.

Q - Terence Flynn {BIO 15030404 <GO>}

Great. Thanks for taking the questions. Maybe two for me. Rob, I appreciate just your recent comments on looking at external innovation. Maybe if you could give us an update on your BD lens, kind of the size and risk profile of potential deals you might consider. I know you did more of the early-stage deals as you walked through at the beginning of the call, but how are you thinking about the opportunities on the forward here?

And then maybe for Roopal, just wondering if you can frame expectations for the upcoming Rinvoq and Luti Phase 3 HS data in terms of what you'd like to see from a target profile perspective. Thank you.

A - Robert A. Michael {BIO 19541407 <GO>}

Hi, Terence. It's Rob. So I'll take your first question here. You're right, I mean, if you look at the last two years, we've invested over \$8 billion in external innovation that have added significant depth to our pipeline with more than 30 deals. And again, these are opportunities that can drive growth in the next decade and beyond.

To recap, I mean, in immunology, we've added new mechanisms, such as TL1A and TREM1 that can play a key role in our combination strategy. We've added oral peptides capabilities from Nimble, the in vivo CAR-T platform from Capstan. In neuroscience, again, our next-generation assets in psychiatry, 932, prednisolone, and emraclidine have all come through BD.

And we acquired a very compelling brain shell platform from Aliada that gives us, I'd say, a very compelling opportunity in Alzheimer's. In oncology, we've added trispecifics and multiple myeloma that can follow etentamig, the in vivo CAR-T platform from Umoja, as well as a PD-1/VEGF antibody, which I highlighted in my remarks, which will complement our ADC portfolio.

And I should also mention the sRNA platform from ADARx can really generate opportunities across all three of those therapeutic areas. I mean, we're clearly focused on early-stage opportunities so that we have solid growth drivers in the middle part of the next decade, given how sizable we expect Skyrizi and Rinvoq to be.

And that's also why we utilize BD to enter into another growth area in obesity, which we will build upon as we identify other differentiated assets. We're clearly taking a close look at what's available out there. And to the extent we see something that we feel is differentiated, we'll pursue it.

And I would say to the extent we see a differentiated asset in any of our core areas, whether early or late-stage, and I think sometimes there's a misconception that we're only interested in early-stage. We are willing to invest in late-stage assets. We certainly have the financial wherewithal.

Strategically, for the company, it's really about positioning the pipeline to deliver that growth in the next decade, because we have a clear line of sight to growth drivers for this decade. But again, given our strong growth outlook and no LOEs this decade, we have plenty of firepower to pursue both early and late-stage opportunities. We're not limited to early-stage, and we're focused on our core areas of immunology, neuroscience, oncology, and building out obesity.

A - Roopal Thakkar {BIO 23690312 <GO>}

Terence, it's Roopal. I'll talk about HS expectations. So, first of all, with Rinvoq, we're excited about this particular study and indication, especially as we look across previous indications.

So, for example, if you look at Crohn's disease and recently alopecia areata, Rinvoq really overperforms relative to any other JAK inhibitor in the class, along with any other biologic, if we were to take a look there. And that's why this study is going to be very important to build on the data that we've generated so far with Rinvoq.

Now, I will say this will be in a treatment failure population, because that's where we believe the JAK inhibitor class will be used in HS, and the dermatologists will already have familiarity with it when they've already used it for atopic dermatitis and soon alopecia areata and vitiligo. So maybe the efficacy won't be as sky high as we would see in a naive population, but we think it can be very meaningful because patients tend to cycle off of these assets as they become available.

Now, with lutikizumab, that one is a different mechanism. It's IL-1 beta, beta in particular is very highly expressed in tissues of patients with hidradenitis suppurativa. And that one will be studied in a naive population and an experienced population. And so far in Phase 2, we've seen amongst the highest efficacy been reported. So hopefully, in Phase 3, we'll see something similar. But you can also follow the positioning of these two future medicines in this space similar to what Jeff was describing previously in IBD.

We'll have a lead asset in the naive setting. This would be lutikizumab. It's also shown very strong safety profile, high efficacy in Phase 2, and then one for those patients that are not finding relief with current biologics or even luti. So our sales teams and medical teams will have both of these to talk about in a similar concept that we have now demonstrated with IBD quite successfully.

A - Elizabeth Shea {[BIO 18811464 <GO>](#)}

Thanks, Terence. Operator, next question, please.

Operator

Next question is from Asad Haider with Goldman Sachs. Your line is open.

Q - Asad Haider {[BIO 6833576 <GO>](#)}

Great. Thanks. Maybe first for Roopal, you talked about this a little bit already, but maybe if you could expand upon how you see the oncology portfolio evolving from here with respect to novel-novel combos. You've got this new externally sourced PD-1/VEGF bispecific that you just brought in alongside your existing ADC asset. Do you see the portfolio as appropriately positioned now from that perspective? And are there other gaps that you see with respect to where the science is evolving?

And then if I could just also throw one in for Scott on the aesthetics franchise. That's been in a protracted downturn based on macro headwinds. I know you've talked about efforts to revitalize growth, looking at TrenibotE approval later this year as a potential catalyst. Are you able to provide any quantitative framing on what you expect to see post-approval and launch, just recognizing that there's some investor debates about the commercial opportunity for that program? Thank you.

A - Roopal Thakkar {[BIO 23690312 <GO>](#)}

Hey, Asad. It's Roopal. I'll start with oncology. You heard us talk about Temab-A and 706 as examples, and Temab-A multiple tumor types. We get rapid efficacy, which is very important, and the strategy being maintaining that efficacy like a chemotherapy or better, but having a better tolerability profile.

For example, comparison to conventional chemo, we don't see the high rates of stomatitis, diarrhea, hair loss, which, in fact, you still see with certain ADCs, but we don't see that profile with our construct, with our linker technology, and our particular Topo warhead, whether that's against c-Met or SEZ6, for example. So, that's a starting point.

And then the combinations become very important to help build on the durability of these assets and utilization over the long-term so the patient is able to benefit maximally. And we believe a PD-1/VEGF is a terrific combo partner. I mentioned colorectal and lung thus far, but as I stated, we're going to see Temab-A readout in head and neck and ovarian. And that could be also potential areas of combination.

The other key component of our oncology strategy is also having the ability to have a biomarker approach where physicians, patients are always seeking an individualized care regimen. And we have seen examples, particularly in GEA, I would say, and other tumor types that when you have slightly increased expression profiles, you see higher efficacy, though that sits up -- sets up an ability to differentiate our portfolio over the long-term. Now, if there's examples where we don't need a biomarker approach, then we won't take it. So, 706 is a good example in small cell lung cancer.

And you asked maybe what's missing. We think a T-cell engager could be a very nice combination with 706, and we have brought one in-house. The other internal development programs and access we already have that we're working on that hopefully we'll get to the clinic soon is in KRAS.

We are interested in lung, colorectal, pancreatic, strong data in pancreatic with Temab-A already, and we feel in the future having a selective KRAS, hopefully that avoids the N and HRAS, would allow for optimal combination. So I would say very excited on the solid front.

And just to briefly touch on, Rob mentioned 969, we haven't talked much about that today. However, we're trying to get that data to ASCO. So I would look for that. That is taking our bispecific technology, similar construct in how we built lutikizumab against IL-1 alpha beta, but in prostate cancer, this would be against PSMA and STEAP, and then we take the warhead and linker technology from Temab-A and add that to 969.

So the team is excited about the potential in prostate cancer, which is a very large tumor type, and I would say with still substantial unmet need, and we described the hem side where we have next-generational BCMA, CD38, and GPRC5D assets moving rapidly into the clinic.

A - Jeff Stewart {[BIO 19485431](#) <GO>}

And then maybe -- it's Jeff. I'll address your TrenibotE and the botox question. I think what's important as we look into 2026, largely what we see is that the sales impact of the TrenibotE will largely accrue really to botox, and I'll walk through that. But given the timing of the launch and the fact that right now we have plans to train, 12,000 core injectors, it's really going to not really appear until towards the end of the year, but it's going to gate heavily into '27.

So I'll give you some sense of how the dynamics will work. We believe when we look at the patient funnel that at peak, we could potentially up to double the inflow of patients that would basically start to move into the toxin market, because there's still -- we look in the U.S. 55 million considers that just haven't made that leap yet. So the first dynamic is the stimulation of the market that starts to -- start to gate in at the end of '26, and certainly, we believe heavily into '27 and '28.

The second thing that we see is that since it is a short-duration toxin, it only lasts for two and a half to three weeks. The real commercial impact goes to your conversion rate, our key performance indicator of how fast we convert to full-strength botox. And the way that we think about that metric is right now we have very high leading share in the U.S. in the low 60s percent.

We would anticipate that once patients start on TrenibotE, as they start to gate in over this time period, that we have a much higher conversion rate than our existing share. So it starts to build share as well. And so net-net, we're very excited about this innovation. We think it's going to operate to sort of increase inflow substantially over our plan, again more in the '27 and beyond standpoint as we get the training up -- ramped up, and also accrue to botox share. So that's how we're looking at that market development over time.

A - Elizabeth Shea {BIO 18811464 <GO>}

Thanks, Asad. Operator, next question, please.

Operator

Next question comes from Vamil Divan with Guggenheim Securities. Your line is open.

Q - Vamil Divan {BIO 15748296 <GO>}

Great. Thanks for taking the question. So maybe one I do have actually still on the botox topic, and then one other one. On botox, we were sort of surprised to see that one selected for the Medicare price reductions in 2028. I'm wondering if that was a surprise to you as well, or you were sort of expecting that.

And then what that might mean for pricing on the cosmetic side? Because I think it's always been tied to therapeutic and cosmetic pricing. So if there's reductions for Medicare on the therapeutic side, what -- do you have to sort of carry that over to cosmetics, or do you maybe have more flexibility in some way in the future?

And then my second question is back on the immunology franchise. And I caught some of what you said around 1Q pricing dynamics, but maybe you can just share your thinking around pricing overall in that market first within Rinvoq first take this year, maybe for the next couple of years, just how things evolve from a pricing perspective? Thank you.

A - Robert A. Michael {BIO 19541407 <GO>}

Thanks, Vamil. It's Rob. Jeff and I will tag team your first question here. So as it relates to botox being selected for Medicare negotiation, we're obviously disappointed that it was selected, given that it's a plasma-derived product and should have been excluded. That said, we did plan conservatively that it could be selected based on CMS spend. So its selection does not impact our long-term growth guidance at all.

A - Jeff Stewart {BIO 19485431 <GO>}

Yes, and also in terms of the pricing separation, we're comfortable with how we understand that effectuation may ultimately take place if and when it does happen in 2028. Certainly, we did see that when the announcement was made, there were comments around Botox Cosmetic, but that's just more of a formulaic dynamic with the way the government looks at NDCs. But it's quite clear that there's a cash pay component. So we don't see a large or meaningful interaction between the two, even if we do see the negotiation take place in '28.

A - Scott T. Reents {BIO 21044531 <GO>}

Sure. And this is Scott. I'll address your question on pricing for Skyrizi and Rinvoq. So we've talked about, and we continue to expect low single-digit pricing headwinds for both those products, both in '26 and over the next few years as well. That's what we've seen.

I would tell you that '25 was unique. We had some pricing tailwinds in '25. And so in the end for Skyrizi, we were roughly flat for pricing on the year for Skyrizi. We had some positive price in the first half of the year, and it was negative price in the back half of the year.

And Rinvoq was slightly down on a year-over-year basis for pricing in '25 as well. Pricing favorability in the first half and pricing unfavorability in the second half netting to slightly down. That does come into play when we look at, for instance, the first quarter of Rinvoq.

You'll see that pricing headwind and frankly, a high single-digit as an unfavorable comparison that Rinvoq is facing on a prior year basis. So those dynamics will be at play, but you're going to see low single-digit pricing in '26 for both products and then going forward as well as our anticipation.

A - Elizabeth Shea {BIO 18811464 <GO>}

Thanks, Vamil. Operator, next question, please.

Operator

Next question is from Steve Scala with TD Cowen. Your line is open.

Q - Steve Scala {BIO 1505201 <GO>}

Thank you so much. It is clear that you are confident in Skyrizi in IBD, but the competition is growing very rapidly. And it looks like it's inevitable it will make an important impact on Skyrizi. So, what can be done to neutralize these gains at this juncture?

And secondly, in the drug pricing deal Abbvie signed with President Trump, the release noted exemption from tariffs and future pricing mandates. Can you elaborate on what exemption from future pricing mandates constitutes beyond avoidance of demonstration projects? And how important is avoidance of demonstration projects? Thank you.

A - Jeff Stewart {BIO 19485431 <GO>}

Yes. Hi, Steve. It's Jeff. Just maybe a few comments. I've outlined how stable our capture rates are. Look, it is a competitive market. There have been changes in terms of frontline and later lines, but we're very, very confident that we're going to see very significant growth and leadership with Skyrizi over IBD as well as the Rinvoq dynamic we discussed.

Now, what are some things that we can think about and are thinking about in terms of continuing the momentum and even increasing our momentum? First, this is not a zero-sum game with two IL-23s. There's a lot of other products in the marketplace. And I think that we've highlighted before we will shortly see a readout over a head-to-head study with Entyvio.

Entyvio is the leading frontline agent or close to it with Skyrizi in UC, and it does pretty well in Crohn's as well. So with the head-to-head data for Skyrizi versus Entyvio, we see a significant opportunity to continue the momentum beyond just the IL-23 growth and the competition that we've been talking about.

The other dynamic that we see there is it's a modest, I would say, market value driver. It gets a lot of play over this idea of a subcutaneous induction versus an IV induction. And as Roopal highlighted in his prepared remarks, we will close that gap, we believe, sometime in early 2027 based on the readout that we're anticipating. So we have many strategies that we continue to pursue to make sure that we can maintain and sustain our Skyrizi leadership over time.

A - Robert A. Michael {[BIO 19541407 <GO>](#)}

And Steve, this is Rob. We're pleased that we were able to reach an agreement with the Trump administration that, again, balances affordability and access and protects the U.S. innovation ecosystem going forward. And so we're pleased where that's landed.

As you noted, similar to many of our peers who did also execute agreements, we are exempt during the term from tariffs, as well as the pricing mandate, inclusive of demonstration projects. So your understanding is correct.

A - Elizabeth Shea {[BIO 18811464 <GO>](#)}

Thank you, Steve. Operator, next question, please.

Operator

Next question is from Mohit Bansal with Wells Fargo. Your line is open.

Q - Mohit Bansal {[BIO 18070890 <GO>](#)}

Great. Thank you very much for taking my question. Maybe a question on the migraine primary care expansion here. So you -- so, I mean, Pfizer yesterday talked about that they are generating about 83% share in the new prescribers for -- in the migraine market.

I'm assuming this is all primary care, but again, would you talk a little bit about how your expansion strategy into primary care going? And then do you -- are you seeing any increased competition there from the competition in the migraine market? Thank you.

A - Jeff Stewart {[BIO 19485431 <GO>](#)}

Yes. Thanks for the question. We're -- as Rob mentioned in one of his responses, we are extremely pleased with our migraine business. I mean, certainly we have a very large botox business. It's the only toxin approved in chronic migraine. And just to be very, very clear, we have the absolute leading brand with Ubrelvy in acute migraine, okay? And that lead is expanding.

We also have recently become the number one branded drug in the episodic oral CGRP for prevention with Qulipta. So we have an extremely strong position. It's very difficult for us to triangulate against what the competitors said, but you can just look at our reported sales when you add up our total oral CGRP sales, which is also rapidly globalizing. So we see, if anything, that we're going to continue to stretch our lead in that area.

Now, we also have a considerable primary care presence right now with both Ubrelvy and Qulipta, where we call on essentially over 70,000 or 80,000 physicians, most of which are primary care physicians. We also cover headache specialists and neurologists, of course. So we have very significant reach, and we have also a very significant lead in this category.

A - Robert A. Michael {[BIO 19541407 <GO>](#)}

And this is Rob. I'll just add. I mean, as you -- we saw the comments as well, it's hard for us to reconcile when you just compare the revenue in 2025 for Qulipta and Ubrelvy combined is almost \$1 billion higher than the competitor. So it's difficult for us to reconcile the numbers that were being quoted.

When I look at the performance of the oral migraine franchise and just the continuous share gains, we're talking about over a point every year and now delivering almost \$3 billion this year, which was, again, the peak that we had previously communicated. Obviously, both brands have a long runway. And so, as I mentioned before, as we look at these brands, we would expect them to exceed \$5 billion now. So just tremendous momentum. The profile of both drugs is very powerful, and the commercial execution has been very strong.

A - Elizabeth Shea {BIO 18811464 <GO>}

Thank you, Mohit.

Q - Mohit Bansal {BIO 18070890 <GO>}

Thank you.

A - Elizabeth Shea {BIO 18811464 <GO>}

Operator, next question, please.

Operator

Next question is from David Amsellem with Piper Sandler. Your line is open.

Q - David Amsellem {BIO 15145021 <GO>}

Hey, thanks. So on Rinvoq and vitiligo, can you talk about the sizing of that opportunity, given the dearth of systemic options? And also, do you perceive a competitive threat from IL-15-directed therapies in vitiligo down the road? That's number one.

And then secondly, as you talk about potential replacements for Vraylar following its LOE, does that also contemplate a long-acting injectable form of cariprazine as one of those replacements? Thanks.

A - Jeff Stewart {BIO 19485431 <GO>}

Yes. Hi. It's Jeff. I'll take the vitiligo question. So, yes, we've highlighted that if you look at our next generation of -- our next wave of Rinvoq approvals, that we anticipate roughly \$2 billion or more in additional peak-year sales, and those are going to start to gate out here relatively soon.

We're right on track with GCA, which is moving quite well. It's the smallest of the bunch. It's difficult to say. When we look at vitiligo, I mean, we've sized that opportunity, roughly just above \$0.5 billion at peak. But again, we could surprise ourselves there. I mean, the data looks quite strong, and to your point, it is the only systemic agent. And so, ultimately, where we end up in terms of that peak-year sales will also depend on the labeling that we get as we go through the process.

Certainly, we're reviewing quite a bit of disease state awareness that will start to gate in with AbbVie spending in the middle part of the year where we're going to basically highlight the fact that there is no systemic treatment for this disease that has tremendous psychosocial impact and is probably still underestimated in terms of what might happen. But again, we're still really roughly

in that \$2 billion-plus range for all of those new indications and excited to bring vitiligo to the market.

A - Roopal Thakkar {BIO 23690312 <GO>}

And David, it's Roopal. Maybe on the competitive side, we'll have to see this over time. I mean, there's growing familiarity with Rinvoq now in dermatology with atopic dermatitis, soon for HS, alopecia areata, and the efficacy is very strong.

And what we're also observing is in alopecia areata and vitiligo seems to continue to increase over time. And the tolerability with our simple oral is also appreciated by physicians and patients. So if that were to enter, again, I think we still have the opportunity to compete based on the high efficacy that we've seen across other indications.

A - Elizabeth Shea {BIO 18811464 <GO>}

Thanks, David. Sorry, sorry, we've got a follow-up on Vraylar, sorry.

A - Roopal Thakkar {BIO 23690312 <GO>}

Yes, and maybe on the Vraylar LOE. Yes, we do have some partnerships in place and then we're assessing, I would just say, not just Vraylar, but other assets in terms of long-acting injectables. We've seen good tolerability and high efficacy with Vraylar. So that one could be quite amenable to this type of strategy.

A - Elizabeth Shea {BIO 18811464 <GO>}

Thanks, David. Operator, next question, please.

Operator

Next question is from Luisa Hector with Berenberg. Your line is now open.

Q - Luisa Hector {BIO 21697697 <GO>}

Hi, there. Thank you. I have just a couple. On aesthetics, just to check, are there any regions where you are losing share in toxins or fillers? And is this franchise [ph] less profitable than when you acquired it? And maybe to touch on obesity, I don't know if you sort of can remind us what the profile is you're looking for in your amylin? And how soon you might pivot into Phase 3? Could you launch on weight loss only without the longer CVOT trials? Thank you.

A - Jeff Stewart {BIO 19485431 <GO>}

Yes. Hi, Luisa. I couldn't understand the full question, but there are some areas where we have had some share loss, I'd say particularly in Brazil. Brazil has been the one area where we've seen some declines in share, particularly in the filler category.

And I'm glad you asked the question because we do have a geographic mix issue that has come in 2025 where Brazil and Latin America tends to grow and we've lost a little bit of share, whereas we have much more stable or growing share in China and Asia. So hopefully, that answers your question.

A - Robert A. Michael {BIO 19541407 <GO>}

And maybe to put a little bit of perspective, just keep in mind that the U.S. and China is a vast majority of the aesthetics business for us. When Jeff talks about a market like Brazil, you're talking about less than \$100 million. So just to put in perspective that, to the extent we have lost some share there, it's a very small market for us.

A - Jeff Stewart {[BIO 19485431 <GO>](#)}

Right.

A - Roopal Thakkar {[BIO 23690312 <GO>](#)}

And then on obesity -- it's Roopal. The key for us there is going to be that tolerability profile along with weight. I think as the incretins read out within a few percentage points of each other, whether it's weekly or monthly, we still feel that there is going to be a substantial number of patients that are going to cycle off of those for variety of reasons, including tolerability.

So with the two Phase 1s that are underway now, us looking for dosing regimens and potentially even going out to monthly is going to be the key for us as we go forward into Phase 2 and Phase 3. With regard to being able to pull in Phase 3, certainly that would be regulatory discussions that we would have, especially if we see strong safety and efficacy profiles as the data emerge.

A - Elizabeth Shea {[BIO 18811464 <GO>](#)}

Thanks, Luisa. Operator, next question, please.

Operator

Next question is from Michael Yee with UBS. Your line is open. And Michael, if you're there, please check your mute button.

Q - Michael Yee {[BIO 20030045 <GO>](#)}

Can you hear me okay? Great. Just in terms immunology --

A - Elizabeth Shea {[BIO 18811464 <GO>](#)}

Yes, we can hear you.

Q - Michael Yee {[BIO 20030045 <GO>](#)}

Yes, in terms of the Crohn's disease immunology platform program that's ongoing, and I think I see on ct.gov, and you've pointed out is critically important, can you just tell me a little bit about what you expect to get from that study? And is the goal to raise the bar in terms of efficacy, and that obviously is the key component of this to extend your leadership? Thank you.

A - Roopal Thakkar {[BIO 23690312 <GO>](#)}

Yes. Hi, it's Roopal. So, yes, the increasing efficacy would be important, the degree of which will be determined by the types of patients. So, if we're studying naive patients, which there could be less and less, as we've seen Skyrizi just have tremendous penetration in IBD in the frontline, then our consideration is what can come after.

So, a key component of these IBD platform studies are also to study patients who have already received Skyrizi. So, there could be a slightly different efficacy outcome there if you're looking at

second and third-line. These are very important lines in IBD because they continue to expand as patients roll off of anti-TNFs, and we've actually seen less and less clinicians turning to anti-TNFs.

So, this is where 23s are becoming very important, as Jeff has highlighted, in particular Skyrizi. And when we look at combinations with Skyrizi, can we treat those patients as well? The tolerability also will be very important. We're doing our best to avoid boxed warnings, if at all possible, which would exist if we combined with an anti-TNF, which is not our strategy because there's less and less utilization.

We do have an alpha-4, beta-7. We've talked about luti. We also have TL1A, which can also be a good combination. The other core component of this strategy is also in parallel, as these data read out, is to look at our ability to co-formulate and deliver these together in a patient-friendly way. So many things coming together, and we're excited to hopefully share some data this year.

Q - Michael Yee {[BIO 20030045 <GO>](#)}

Thank you.

A - Elizabeth Shea {[BIO 18811464 <GO>](#)}

Thanks, Michael. Operator, we have time for one final question.

Operator

The final question is from Evan Seigerman with BMO Capital Markets. Your line is open.

Q - Malcolm Hoffman {[BIO 24178091 <GO>](#)}

Hi. This is Malcolm Hoffman on for Evan. Thanks for taking our question. So I wanted to touch on 932, which I think has been addressed or at least mentioned a couple times throughout the call. But I know you have the Phase 2 readout later this year in bipolar. As a follow-on to Vraylar, can you maybe give us some framing on how you're thinking this could compare to Vraylar, given the heavier targeting of D3 versus D2?

And then just a second follow-up there. If you're looking to pursue drug anxiety as well. Is there any expectation to expand into areas like addiction, again, given the heavier D3 reliance here? Thank you.

A - Roopal Thakkar {[BIO 23690312 <GO>](#)}

Thanks. It's Roopal. I'll take that. So, yes, later this year we'll see the data of 932. Currently, I'll talk about generalized anxiety disorder. We do not have that approved for Vraylar, though. We have seen effect in patients who have some anxiety, and we keep lowering of those scores. And that's why I think we see such strong uptake of Vraylar and why it was important to study a next-generational version.

Hopefully, with less D2, we anticipate potentially less movement disorder. So that's one thing that we would be looking for is that safety and tolerability profile, while maintaining efficacy. So I think that will be important. We'll also see a breakdown between Bipolar 1 and Bipolar 2. We have observed that Vraylar has worked better on the Bipolar 1 side. That could be a potential outcome, but that's data that we're going to look at. Also, that will be very important.

And in terms of other indications, that could be, what you mentioned, could be something else that we would look at. And as we step back and consider all of psychiatry, we'll have two studies

in Phase 2 with a kappa opioid receptor antagonist, which we believe has strong potency. I mentioned brexisiloin in depression, but you could imagine there could be indication expansion for that one as well.

And as we make good progress with emraclidine and those doses being able to go higher, again having potential there in psychosis of neurodegeneration as well as schizophrenia that we've mentioned. So along with 932, there's a whole portfolio there. And I'll mention we continue to partner with Gedeon Richter and there's even a more D3-leaning asset that is within preclinical talks that could also see a first-in-human within the next year or 18 months.

A - Elizabeth Shea {[BIO 18811464](#) <GO>}

Thanks, everyone, and that concludes today's conference call. If you'd like to listen to a replay of the call, please visit our website at investors.abbvie.com. Thanks again for joining us.

Operator

This concludes today's call. Thank you for your participation. You may disconnect at this time.

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